

1. Administrative & Study Identification

Full Study Title: Real-World Effectiveness of High-Dose Tafamidis on Neurologic Disease Progression in Mixed-Phenotype Transthyretin Amyloid Cardiomyopathy
Short Title/Acronym: Tafamidis Real-World Effectiveness in Mixed-Phenotype ATTR-CM
Protocol Number: B3461121
NCT Number: NCT06393465
Posting ID: 1127709698059579698 **Trial Status:** ACTIVE_NOT_RECRUITING **Sponsor/Company Name:** Pfizer
Investigational Product: Tafamidis (VYNDAMAX 61 mg or VYNDAQEL 80 mg)
Phase of Development: Observational (Not Applicable)
Medical Monitor: Pfizer Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To describe and compare the rate of neurologic disease progression before and after the initiation of tafamidis in patients with mixed-phenotype ATTR-CM receiving tafamidis 61 mg daily in a real-world setting.
Secondary Objectives: To examine overall real-world clinical effectiveness and general outcomes utilizing data existing within patient medical records.

2.2 Background & Rationale To examine the clinical effectiveness of Tafamidis in patients with Mixed Phenotype Transthyretin Amyloidosis. This study leverages retrospective data existing in patient medical records to address the gap in real-world evidence regarding the impact of high-dose tafamidis specifically on neurologic disease progression in this mixed-phenotype cohort.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Cohort) **Control Type:** Single-arm (Retrospective pre- vs. post-treatment comparison) **Blinding:** Open-label / Non-blinded **Randomization:** N/A (Retrospective Cohort)

3.2 Planned Enrollment & Duration Target \$N\$: 50 patients **Number of Sites:** 4 study sites (MedStar in the U.S., and three other sites located in Canada, Portugal, and Italy) **Estimated Study Start:** 15-Jun-2024 **Estimated Primary Completion:** 13-Mar-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years at diagnosis. 2. Diagnosed with ATTRv-CM or ATTRwt-CM, mixed phenotype. 3. Treated with tafamidis, as VYNDAMAX 61 mg (one 61-mg tafamidis capsule) orally once daily for ≥ 12 months or started on tafamidis 20 mg then switched to VYNDAMAX 61 mg for ≥ 12 months. 4. Have had ≥ 1 pre- and ≥ 1 post-treatment neurologic assessments.

4.2 Exclusion Criteria 1. History of any organ transplant. 2. Individuals who are non-ambulatory. 3. Prior or current treatment with any disease-modifying therapy (investigational or approved) alone or in combination, except tafamidis, as VYNDAQEL 80 mg [four 20-mg tafamidis meglumine capsules] orally once daily or VYNDAMAX 61 mg [one 61-mg tafamidis capsule] orally once daily. 4. Peripheral neuropathy attributed to causes other than ATTR amyloidosis (e.g., diabetes mellitus, B12 deficiency, hypothyroidism, shingles, Lyme disease, HIV infection, secondary to injury, chronic kidney disease). 5. Patient's data fails to pass data quality checks.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Tafamidis as VYNDAQEL 80 mg (four 20-mg capsules) once daily or VYNDAMAX 61 mg (one 61-mg capsule) once daily. Route of Administration: Oral Duration of Treatment: At least 12 months of prior treatment (retrospectively evaluated).

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant therapies for heart failure and related comorbidities not affecting ATTR progression. Prohibited: Prior or current treatment with any other disease-modifying therapy (investigational or approved).

6. Study Timeline & Visit Schedule

(Adapted for Retrospective Observational Protocol) | Visit ID | Window | Key Activities/Procedures | | :--- | :--- | :--- | | Baseline | Pre-Initiation | Medical History, Pre-treatment neurologic assessment | | Follow-up | ≥ 12 months Post-Initiation | Medical record review, Post-treatment neurologic assessment |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Rate of neurologic disease progression before and after initiation of tafamidis 61 mg daily. Measurement Method: Retrospective analysis of documented

neurologic assessments in patient medical records from baseline through at least 12 months of treatment.

7.2 Secondary & Exploratory Endpoints 1. Change from Baseline in modified Body Mass Index (BMI). Safety and tolerability endpoints as recorded through standard real-world data collection.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Extracted retrospectively from electronic medical records (EMR) and hospital databases. **Serious Adverse Events (SAEs):** Reported according to observational real-world evidence (RWE) guidelines. **Data Monitoring Committee (DMC):** Quality checks will be performed on patient data before cohort inclusion.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Observational Cohort (Patients meeting all eligibility and data quality criteria). **Sample Size Justification:** $N = 50$ (Sufficient to perform descriptive and comparative analyses within the specific mixed-phenotype cohort). **Handling of Missing Data:** Patients whose data fails to pass fundamental data quality checks or lacking paired neurologic assessments will be excluded from the analysis.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study complies with Good Clinical Practice (GCP), the Declaration of Helsinki, and relevant patient data privacy laws (e.g., HIPAA) regarding retrospective medical chart reviews. **Monitoring Plan:** Centralized data review focusing on ensuring data completeness and accuracy from extracted patient records. **Audit Trail:** Standard database lock procedures for observational analyses.

11. Study Identifiers & Governance

Primary ID: B3461121 **Registry Number:** NCT06393465 **Posting ID:** 1127709698059579698 **Sponsor/Company:** Pfizer **Study Title:** Real-World Effectiveness of High-Dose Tafamidis on Neurologic Disease Progression in Mixed-Phenotype Transthyretin Amyloid Cardiomyopathy **Official Regulatory Title:** Real-World Effectiveness of High-Dose Tafamidis on Neurologic Disease Progression in Mixed-Phenotype Transthyretin Amyloid Cardiomyopathy (ATTR-CM) **Preferred UMLS Name:** Transthyretin Amyloid Cardiomyopathy **Drug Name:** Tafamidis **ATC Code:** N07XX08

Semantic Type: Disease or Syndrome **MeSH Code:** D009202 **HPO Code:** HP:0001638 **SNOMED ID:** 85898001

12. Clinical Framework (ATTR-CM Specific)

Primary Condition: Transthyretin Amyloidosis Cardiomyopathy (ATTR-CM) **Diagnosis Threshold:** Confirmed ATTRv-CM or ATTRwt-CM, mixed phenotype **Medical Methodology:** Disease-modifying therapy (Transthyretin Stabilization) **Optimization Target:** Mitigation/Delay of Neurologic Disease Progression

13. Study Procedures & Methodology

Management Pathway: Retrospective analysis of real-world clinical records **Education Protocol (HCP):** N/A (Retrospective observational design) **Education Protocol (Patient):** N/A (Retrospective observational design) **Quality Audit Mechanism:** Mandatory patient data quality checks prior to data extraction and endpoint analysis

14. Observation & Follow-Up Schedule

Total Duration: ≥ 12 months of treatment observation **Onsite Visit Cadence:** N/A (Medical records review) **Remote Monitoring Frequency:** N/A (Retrospective design) **Checklist Review Interval:** Routine EHR data extraction

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]						
Statistician	[Name]	_____	[DD-MMM-YYYY]						